This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practise under it.

Healthcare professionals must work within their own competence. This PGD does not remove professional obligations or accountability.

ISSUED, VALID FROM 30 SEPTEMBER 2026

Patient Group Direction

for the administration of Gardasil 9 (Human Papillomavirus 9-valent Vaccine) for the prevention of HPV-related cancers and genital warts

Prevention of HPV-related cancers and genital warts

From age 9 years onwards, all sexes.

Patient Group Direction, version 008, issued 30 September 2026. Gardasil 9, intramuscular, from 9 years of age, all sexes.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

What this PGD is for

The administration of the HPV vaccine to a patient aged 9 years or over, of any sex, who wants protection against the cancers and genital warts caused by human papillomavirus, by a registered pharmacist or registered pharmacy technician in a community pharmacy, without a prescription.

What it authorises

- Gardasil 9 suspension for injection, 0.5 mL by the intramuscular route: one dose under 25 years of age, two doses from 25, and three doses where the patient is immunosuppressed or living with HIV.

What it does not cover

- A patient who has had a confirmed anaphylactic reaction to a previous dose or to any component of the vaccine.
- A patient who is acutely unwell with a fever. Postpone.
- Children under 9.

Summary of Green Book guidance for HPV vaccination

Immunisation against infectious disease (the Green Book), chapter 18a, Human papillomavirus (HPV), UK Health Security Agency, dated 19 June 2023 (GOV.UK last updated 20 June 2023). Summarised 9 September 2026.

What HPV vaccination is for, and who the UK programme covers

HPV is a double stranded DNA virus. Genital infection is associated with genital warts and anogenital cancers in both men and women, and persistent high-risk HPV infection is detectable in more than 99% of cervical cancers and is causally linked to cancers of the vulva, vagina, penis, anus and some head and neck cancers.

HPV vaccination is routinely recommended for all girls and boys from 11 years of age, offered in school year 8 in England and Wales, S1 in Scotland and school year 9 in Northern Ireland. The programme was extended from girls only to include adolescent boys from September 2019.

Males and females in cohorts eligible for the national programme remain eligible until their 25th birthday. Those coming to the UK from overseas and registered with a GP practice should be offered vaccination if they are aged under 25.

All gay, bisexual and other men who have sex with men (GBMSM) aged up to and including 45 years who attend specialist sexual health services or HIV services are eligible if not already vaccinated. Anyone eligible under that programme who started but did not complete the schedule before reaching 46 should complete the course.

Vaccination may also be offered to individuals attending specialist sexual health or HIV services who were not eligible for the routine programme and are deemed to have a similar risk profile, including some transgender individuals, sex workers, and men and women living with HIV.

The vaccines are licensed from nine years old, but vaccination of children aged 9 to 11 is not covered by the national programme.

Prevention advice should include safer sex, and all women, whether vaccinated or not, should be strongly encouraged to attend routine cervical screening at the scheduled age.

The vaccine, the dose, and the schedule by age and immune status

Gardasil 9 contains virus-like particles for nine HPV types (6, 11, 16, 18, 31, 33, 45, 52 and 58) adsorbed on amorphous aluminium hydroxyphosphate sulphate adjuvant. It contains no thiomersal, no viral DNA and no live organisms, and cannot cause the diseases against which it protects.

Since July 2022 Gardasil 9 is the only HPV vaccine supplied for the UK national programme, for both the adolescent and the GBMSM cohorts.

Each dose is 0.5 mL given intramuscularly into the upper arm or anterolateral thigh. Localised reactions are more common when these vaccines are given subcutaneously.

UNDER 25 AT VACCINATION: JCVI recommends a single dose schedule for all HPV vaccines. Anyone immunocompetent who received one dose before reaching 25 does not require any further doses. The three dose schedule below applies to immunosuppressed and HIV positive individuals whatever their age.

25 AND OVER: JCVI recommends two doses at 0 and 6 to 24 months, so the second dose is given six to 24 months after the first. Any gap between 6 and 24 months is clinically acceptable, and for Gardasil 9 the minimum interval can be 5 months.

IMMUNOSUPPRESSED OR KNOWN HIV POSITIVE, including those on antiretroviral therapy: a three dose schedule, because there is limited data on fewer than 3 doses in these groups. For Gardasil 9 that is 0, 1 and 4 to 6 months, with a second dose at least one month after the first and a third at least three months after the second, ideally all within 12 months.

Eligible GBMSM known to be HIV positive should be offered the vaccine regardless of CD4 count, antiretroviral therapy use or viral load.

An interrupted course should be resumed and not repeated, even if more than 24 months have elapsed. Where a dose has inadvertently been given early, discount it and repeat it once the recommended interval has elapsed and at least 4 weeks after the dose given in error.

Evidence supports the interchangeability of all HPV vaccines, so someone who started with a product not available in the UK follows a mixed schedule.

HPV vaccine can be given at the same time as other vaccines including Td/IPV, MMR, influenza, MenACWY and hepatitis B, at a separate site, preferably in a different limb, and if in the same limb at least 2.5 cm apart. Record the site of each.

Contraindications and precautions, and what is NOT a contraindication

There are very few individuals who cannot receive HPV vaccine. Where there is doubt, seek advice from the relevant specialist or the local immunisation or health protection team rather than withholding vaccination.

Do not give the vaccine to anyone who has had a confirmed anaphylactic reaction to a previous dose of HPV vaccine, or to any component of the vaccine.

YEAST ALLERGY IS NOT A CONTRAINDICATION. Gardasil and Gardasil 9 are grown in yeast cells, but the final product does not contain yeast as an excipient or ingredient and at most contains trace amounts of yeast protein below 0.007 micrograms.

ROUTINE QUESTIONING ABOUT LAST MENSTRUAL PERIOD AND PREGNANCY TESTING IS NOT REQUIRED before offering HPV vaccine.

Minor illness without fever or systemic upset is not a reason to postpone. If the individual is acutely unwell, postpone until recovery so as not to confuse the differential diagnosis of the acute illness.

Individuals with bleeding disorders may be vaccinated intramuscularly if, in the opinion of a doctor familiar with their bleeding risk, small volume intramuscular injections can be given with reasonable safety, and vaccination can be scheduled shortly after treatment such as haemophilia treatment.

Individuals on stable anticoagulation, including those on warfarin who are up to date with INR testing and whose latest INR was below the upper threshold of their therapeutic range, can be vaccinated intramuscularly with a fine needle (23 gauge or finer) followed by firm pressure without rubbing for at least 2 minutes.

Suboptimal immunogenicity has been observed in transplant patients, so additional doses should be considered after treatment is finished or recovery has occurred, and specialist advice may be required.

Pregnancy and breastfeeding

There is no known risk in giving inactivated or recombinant vaccines during pregnancy or while breastfeeding. Inactivated vaccines cannot replicate and so cannot cause infection in the mother or the fetus.

Clinical development data on almost 4,000 reported pregnancies showed no significant difference in types of anomaly or in the proportion of pregnancies with an adverse outcome, and a nationwide Danish register cohort found no significant risk of spontaneous abortion, major birth defect, stillbirth, preterm birth, small size for gestational age or low birth weight.

Individuals known to be sexually active, including those who are or have been pregnant, may still be susceptible to high-risk HPV infection and could benefit from vaccination.

If a woman finds out she is pregnant after starting a course, termination of pregnancy following inadvertent immunisation should NOT be recommended.

Adverse reactions, including anxiety-related syncope

The commonest reaction is mild to moderate short-lasting pain at the injection site. An immediate localised stinging sensation and redness have also been reported.

Other common reactions are headache, myalgia, fatigue and low-grade fever.

Syncope (vasovagal reaction) can occur during any vaccination and is commonest among adolescents and adults, and some individuals experience panic attacks before vaccination. Fainting and panic attacks occurring before or very shortly after vaccination are events associated with the injection process rather than direct reactions to the vaccine.

Anaphylaxis is a very rare recognised side effect of most vaccines. Green Book chapter 8 sets out how to distinguish faints and panic attacks from anaphylaxis.

Report suspected anaphylaxis meeting the clinical features in chapter 8 via the Yellow Card Scheme as anaphylaxis. Report less severe allergic reactions as allergic reaction rather than anaphylaxis.

Population studies from the UK, Norway, Finland and the Netherlands have found no evidence that HPV vaccines cause chronic fatigue syndrome, and national and international safety committees have concluded that reported concerns about a range of chronic syndromes are unfounded.

Storage, presentation and disposal

Store in the original packaging at +2C to +8C, ideally aiming for 5C, protected from light.

Effectiveness cannot be guaranteed unless the vaccine has been stored at the correct temperature. Heat speeds the decline in potency and reduces shelf life. Freezing may cause increased reactogenicity and loss of potency, and can cause hairline cracks in the container leading to contamination.

HPV vaccines are supplied as suspensions of virus-like particles in pre-filled syringes. A white precipitate may develop during storage, so shake before use to form a white cloudy liquid.

Dispose of used vials, ampoules and syringes in a puncture-resistant sharps box according to local authority regulations and Health Technical Memorandum 07-01, Safe management of healthcare waste.

The Patient Group Direction

How to use this PGD

Read this first: three points that are easy to get wrong

- THIS PGD COVERS ALL SEXES. HPV vaccination is for boys and men as much as for girls and women. The GBMSM cohort, which national policy singles out for protection up to 45 years of age, is male by definition. Any suggestion that this is a service for females only is wrong.
- YEAST ALLERGY IS NOT A CONTRAINDICATION. Green Book chapter 18a states this in terms: Gardasil 9 is grown in yeast cells but the finished vaccine contains no yeast as an ingredient, at most trace protein below 0.007 micrograms. Both were wrong and both are corrected.
- PREGNANCY: DO NOT ASK ABOUT PERIODS AND DO NOT PREGNANCY TEST. Green Book chapter 18a states that routine questioning about last menstrual period and pregnancy testing is not required before offering HPV vaccine. Vaccination is postponed in a woman known to be pregnant because the SPC says the data are insufficient, not because harm has been shown. If a patient turns out to have been pregnant when vaccinated, that is not a cause for alarm and termination must not be recommended.

The schedule depends on age and immune status, and most of it is off-label

- IMMUNOCOMPETENT and UNDER 25 at the time of vaccination: ONE dose. That is the whole course. Do not book further doses.
- 25 AND OVER: TWO doses, the second 6 to 24 months after the first. The minimum interval for Gardasil 9 is 5 months.
- IMMUNOSUPPRESSED OR KNOWN HIV POSITIVE, any age: THREE doses at 0, 1, and 4 to 6 months, ideally completed within 12 months.
- THE ONE DOSE SCHEDULE AND THE TWO DOSE SCHEDULE ARE OFF-LABEL. The Gardasil 9 SPC contains no one dose schedule at all, and for anyone aged 15 or over at first injection it specifies three doses. The UK one and two dose schedules come from JCVI, not from the product licence. Consent to off-label use must be obtained and recorded. See the row below.
- You must therefore ask about immunosuppression and HIV status before deciding the schedule.
- An immunocompetent person who had one dose before their 25th birthday needs no further doses, whatever their age now. A person who is immunosuppressed or known HIV positive completes the three dose course whatever their age at the first dose: a single dose given before 25 is counted as the first dose, not repeated, and the remaining doses are given at the three dose intervals.
- Observe every patient for 15 minutes after vaccination, seated.

On charging for doses the patient does not need

This matters commercially as well as clinically. Getting the schedule right is not only a safety point. Where a patient has already been given and charged for surplus doses under the old schedule, that is a matter for the pharmacy to put right with them directly.

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Healthcare professionals must work within their own competence. This PGD does not remove professional obligations or accountability.

Patient Group Direction

for the administration, for the prevention of HPV-related cancers and genital warts, of:

Gardasil 9 for vaccination against human papillomavirus

Patient Group Direction for the administration of Gardasil 9 suspension for injection by the intramuscular route, for the prevention of HPV-related disease, in individuals of any sex aged 9 years and over.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC.
- All users must be trained to the National Minimum Standards and Core Curriculum for Immunisation Training, and be competent in intramuscular vaccination technique.
- All users must be competent in the recognition and immediate management of anaphylaxis, and hold current basic life support training.
- All users must be competent in assessing consent in children and young people, including parental responsibility and Gillick competence. This PGD starts at 9 years of age and the largest single cohort is 12 to 13 year olds, so children are squarely in scope.
- All users must be able to select the correct schedule from age and immune status, and must understand that the one dose and two dose schedules are off-label and require documented consent.
- All users must be able to manage a request from a male patient, including a GBMSM patient, without embarrassment or assumption, and must understand that this vaccine is not a female-only product.
- All users must be competent in vaccine handling and cold chain management, including the action to take on a cold chain excursion.
- All users must be familiar with the current SPC for Gardasil 9, Green Book chapter 18a, and Green Book chapter 8 on the management of anaphylaxis and the distinction between a faint, a panic attack and anaphylaxis.
- All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required, must work in compliance with the SOPs of their own employer, must hold appropriate indemnity, and must practise only within the bounds of their own competence.
- All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so.
- All users must be familiar with the current Summary of Product Characteristics for every product named in this PGD, and with current BNF and national guidance for the condition.
- All users must be up to date with MHRA drug safety alerts and recalls relevant to these products.
- All users must be up to date with their CPD requirements and appraisal.
- All users must hold appropriate professional indemnity covering this service.
- All users must understand capacity and consent, including the Mental Capacity Act 2005, and must be able to assess consent in children and young people where this PGD covers them.

The PGD

Indication	Active immunisation of individuals from 9 years of age against premalignant lesions and cancers of the cervix, vulva, vagina and anus caused by vaccine

	HPV types, and against genital warts caused by HPV types 6 and 11. Gardasil 9 protects against HPV types 6, 11, 16, 18, 31, 33, 45, 52 and 58. It is prophylactic only: it has no effect on an established infection or on existing disease, and it is not a treatment.
Inclusion criteria	• Aged 9 years and over. There is no upper age limit in the marketing authorisation. Individuals aged 9 to 11 are licensed but fall outside the national programme, so a private supply is the only route available to them. • **ANY SEX. Male, female, transgender and non-binary individuals are all in scope. The GBMSM cohort is specifically recognised in national policy up to and including 45 years of age.** • **Requesting vaccination privately, and made aware that they may be entitled to receive the vaccine free of charge from an NHS service. NHS eligibility is set out in the appendix. Where the patient is NHS-eligible, tell them so and record that you did.** • **Immune status established, so that the correct schedule can be selected. Ask directly whether the individual is immunosuppressed or known to be living with HIV.** • **Valid informed consent obtained. Where the individual is under 16, from a person with parental responsibility, or from the young person where assessed as Gillick competent, with the basis of that assessment recorded.** • **Consent to off-label use obtained and recorded where the one dose or the two dose schedule is being used. See the row below.** • No exclusion criterion present.
The UK schedules are off-label: what to say and record	**The Gardasil 9 SPC provides only two schedules: a two dose 0 and 6 to 12 month course for individuals aged 9 to and including 14 at first injection, and a three dose 0, 2 and 6 month course for individuals aged 15 and over at first injection. It contains no one dose schedule for anybody.** **It follows that:** • The single dose given to an immunocompetent individual under 25 is off-label in every case. • The two dose course given from 25 years of age is off-label, because the SPC requires three doses from 15 years. • The three dose 0, 1 and 4 to 6 month course sits within the SPC minimum intervals and is not off-label on that ground. **Green Book chapter 18a recommends these schedules but does not itself flag their licensing status. This PGD states it plainly so that nobody administers an off-label schedule believing it to be licensed.** **Where an off-label schedule is used, tell the patient all of the following and record that you did:** • That the number of doses being given follows current UK national recommendations from the Joint Committee on Vaccination and Immunisation. • That this differs from the manufacturer's licence, which specifies more doses. • That the recommendation reflects evidence published since the licence was granted showing that fewer doses give comparable protection in this age group. • That they may choose the licensed schedule instead if they prefer, and what that would involve. **Record the consent as given for off-label use, naming the schedule. A general consent to vaccination is not sufficient.**
Exclusion criteria	**Refer, do not vaccinate, where any of the following applies.**

	• Confirmed anaphylactic reaction to a previous dose of any HPV vaccine. • Confirmed anaphylactic reaction to any component of Gardasil 9, or hypersensitivity following previous administration of Gardasil 9 or Gardasil / Silgard. • Acute severe febrile illness or acute systemic upset. Postpone until fully recovered. A minor illness without fever is not a reason to defer. • **Known to be pregnant. Postpone until after the pregnancy. Do not question about last menstrual period and do not pregnancy test, in line with Green Book chapter 18a.** • **Has already completed a full course of HPV vaccine appropriate to their age and immune status. For an immunocompetent individual this includes anybody who received a single dose before their 25th birthday. An immunosuppressed or HIV positive individual has completed the course only after three doses, whatever their age at the first dose.** • **Under 16 and valid consent cannot be obtained from a person with parental responsibility, and the young person is not assessed as Gillick competent.** • Consent to off-label use not obtained, where an off-label schedule would be used. **YEAST ALLERGY IS NOT AN EXCLUSION. Vaccinate. Green Book chapter 18a is explicit that yeast allergy does not contraindicate HPV vaccine.**
Cautions	**Immunosuppression and HIV. Not a reason to withhold the vaccine, but it changes the schedule to three doses, whatever the patient's age at the first dose and even where a single dose was given before the 25th birthday: that dose is counted and the remaining doses are given. Eligible GBMSM known to be HIV positive should be offered the vaccine regardless of CD4 count, antiretroviral therapy or viral load. The response may still be suboptimal, and in transplant recipients additional doses may be considered after treatment finishes; that is a specialist decision, not one for this PGD.** Bleeding disorders, thrombocytopenia and anticoagulation. Intramuscular vaccination is generally acceptable. Use a 23 gauge or finer needle and apply firm pressure without rubbing for at least two minutes. For a patient on treatment for haemophilia, schedule shortly after that treatment. A patient on stable warfarin who is up to date with INR testing and whose latest INR is below the top of their therapeutic range may be vaccinated intramuscularly. If in any doubt, contact the clinician responsible for the anticoagulation. Breastfeeding. Gardasil 9 can be used during breastfeeding. Not a reason to defer. **Syncope. Fainting, sometimes with tonic-clonic movements on recovery, can occur before or after any injection and is commonest in adolescents. It is a response to the needle, not to the vaccine. Vaccinate seated and have procedures in place to prevent injury from a faint.** This vaccine will not protect against every HPV type and does not clear an existing infection. It is not a substitute for cervical screening, and it is not a substitute for barrier protection against sexually transmitted infection. Complete a course with Gardasil 9 wherever possible. Where an individual started a course overseas with a different HPV vaccine, evidence supports interchangeability and a mixed schedule may be completed. Immunoglobulin or blood products within the previous three months. Not studied. Not a contraindication, but record it.
Anaphylaxis and	**Adrenaline (epinephrine) 1 in 1,000 injection must be immediately**

observation	**available in the room whenever a vaccine is administered under this PGD, in date, together with a written anaphylaxis protocol consistent with current Resuscitation Council UK guidance, and a telephone.** All staff administering vaccines must be trained in the recognition and immediate management of anaphylaxis and must hold current basic life support training. **OBSERVE EVERY PATIENT FOR 15 MINUTES AFTER VACCINATION, SEATED. The Gardasil 9 SPC specifies approximately 15 minutes because of syncope.** Be able to distinguish a faint and a panic attack from anaphylaxis. Green Book chapter 8 sets out the clinical features of each. A faint after an injection in an adolescent is common; anaphylaxis is very rare.
Actions if the patient is excluded or declines	● Discuss the reason with the patient and make sure they understand it. ● **Where the patient is NHS-eligible, direct them to the NHS route rather than simply declining. Adolescents are vaccinated in school; GBMSM up to 45 are vaccinated at sexual health and HIV services.** ● Where vaccination is postponed for pregnancy or acute illness, agree when they should come back and say so clearly. A postponement is not a refusal. ● Where the exclusion is a previous anaphylactic reaction, refer to the GP or an allergy service; do not simply turn the patient away. ● Document the reason for exclusion, the advice given and the decision reached. Inform or refer to the GP as appropriate.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP as appropriate. Anaphylaxis after any vaccine should be reported even where the diagnosis is uncertain.
Records to be kept	● **That valid informed consent was given, and from whom. Where the individual is under 16, the name and relationship of the person with parental responsibility, or the basis of the Gillick assessment.** ● **Where an off-label schedule was used, that consent to off-label use was given, naming the schedule.** ● Patient name, address, date of birth, and the GP with whom they are registered. ● **Immune status as established at the consultation, because it determines the schedule.** ● **Vaccine name and batch number, and expiry date. The SPC requires the name and batch of any biological product to be recorded for traceability.** ● **Which schedule was selected and the reason, being the patient's age at first dose and their immune status.** ● **Dose number in the course, and the date the next dose is due, or that no further dose is required.** ● Date of administration, dose volume, route and anatomical site. Where another vaccine was given at the same visit, the site of each. ● **That the patient was told whether they are eligible for NHS vaccination.** ● Name and registration number of the healthcare professional administering. ● **That the 15 minute observation period was completed.** ● Advice given, including advice given if the patient is excluded or declines. ● Details of any adverse drug reactions and the actions taken.

	● That the vaccine was administered under this PGD. Records signed, dated, legible and contemporaneous. Adults 18 and over: 8 years. Under 18s: until the 25th birthday, or the 26th where the patient was 17 when the course finished. Give the patient a written record of the dose and, where applicable, the date the next one is due.

The medicine

Name, form and strength	● GARDASIL 9 suspension for injection, or Gardasil 9 suspension for injection in a pre-filled syringe. Merck Sharp and Dohme (UK) Limited. PLGB 53095/0023. ● 0.5 mL per dose, containing L1 protein virus-like particles of HPV types 6, 11, 16, 18, 31, 33, 45, 52 and 58, adsorbed on amorphous aluminium hydroxyphosphate sulfate adjuvant. ● **Produced in Saccharomyces cerevisiae. The finished vaccine contains no yeast as an excipient. Yeast allergy is not a contraindication.** ● Excipients: sodium chloride, histidine, polysorbate 80, borax, water for injections. Essentially sodium-free. Thiomersal free. ● Gardasil 9 is the only HPV vaccine supplied for the UK national programme.
Legal category	POM.
Dose and frequency	**SELECT THE SCHEDULE FROM AGE AT FIRST DOSE AND IMMUNE STATUS.** ● **IMMUNOCOMPETENT, UNDER 25 at vaccination: ONE dose of 0.5 mL. Course complete. Off-label.** ● **IMMUNOCOMPETENT, 25 AND OVER: TWO doses of 0.5 mL, the second 6 to 24 months after the first. Minimum interval for Gardasil 9 is 5 months. Any gap between 6 and 24 months is clinically acceptable. Off-label.** ● **IMMUNOSUPPRESSED OR KNOWN HIV POSITIVE, any age: THREE doses of 0.5 mL at 0, 1, and 4 to 6 months. All three ideally within 12 months. This applies whatever the patient's age at the first dose, including a patient who received a single dose before their 25th birthday, who completes the remaining doses.** ● An immunocompetent person who received a single dose before their 25th birthday requires no further doses. An immunosuppressed or HIV positive person who received a single dose at any age completes the three dose course: give the second dose now and the third at least three months after it. ● Interrupted course: resume, do not repeat, even where more than 24 months have elapsed. Allow the appropriate interval between the remaining doses. ● **Dose given too early in error: discount it, and repeat once the correct interval has elapsed and at least 4 weeks after the dose given early.** ● On the three dose course, where the second dose was late and the patient is unlikely to return after three months, the third dose may be given at least one month after the second. ● The need for a booster has not been established. Do not offer one.
Quantity to be administered	One 0.5 mL dose per patient per attendance, in accordance with the schedule above. This PGD does not permit supply of vaccine to the patient for administration elsewhere.
Route and method of	Intramuscular. Preferred site the deltoid area of the upper arm, or the higher

administration	anterolateral thigh. Never intravascularly, subcutaneously or intradermally, and never mixed in a syringe with another vaccine. Shake well before use: the suspension is a clear liquid with a white precipitate before agitation and a white cloudy liquid after. Inspect for particles and discolouration and discard if either is present. Administer as soon as possible after removal from the refrigerator. Gardasil 9 may be given at the same visit as other vaccines including Td/IPV, MMR, influenza, MenACWY and hepatitis B, at a separate site, preferably a different limb, or at least 2.5 cm apart, recording the site of each.
Storage and cold chain	Store at +2C to +8C in the original outer carton to protect from light. Do not freeze. Discard if frozen. **COLD CHAIN EXCURSION: the SPC states that the vaccine components are stable for 96 hours at 8C to 40C, or for 72 hours at 0C to 2C, after which the vaccine must be used or discarded. That figure guides a temporary excursion only. Any stock exposed outside +2C to +8C must be quarantined and risk assessed in accordance with UKHSA Vaccine Incident Guidance before further use. Do not administer excursion stock until that assessment is complete.**
Disposal	Dispose of used syringes, needles, vials and any unused vaccine in a UN-approved puncture-resistant sharps container in accordance with local authority requirements and HTM 07-01, Safe management of healthcare waste.
Adverse effects	Very common: injection site pain, swelling and erythema; headache. Common: pyrexia, fatigue, dizziness, nausea, injection site pruritus and bruising. Uncommon: lymphadenopathy, syncope sometimes with tonic-clonic movements, vomiting, urticaria, arthralgia, myalgia, asthenia, chills, malaise, injection site nodule. Rare: hypersensitivity. Not known: anaphylactic reactions. Reported after the related quadrivalent vaccine: injection site cellulitis, idiopathic thrombocytopenic purpura, anaphylactoid reactions, bronchospasm, acute disseminated encephalomyelitis and Guillain-Barre syndrome, with causality not established. Consult the current SPC.

Information for the patient

Written information	Supply the patient information leaflet provided with the vaccine, and a written record showing the vaccine name, batch number, date, site, dose number and either the date the next dose is due or a clear statement that the course is complete.
Counselling	● **If you are under 25 and not immunosuppressed, one dose is the whole course. You do not need to come back. This is current national policy and it is not us cutting corners.** ● This vaccine protects against the HPV types that cause most cervical and other anogenital cancers, most HPV-related throat cancers, and most genital warts. ● It works best before you are exposed to the virus, but it is still worth having later. ● **It cannot treat an infection you already have, and it will not clear existing warts or abnormal cells.** ● **Keep going to cervical screening when you are invited. The vaccine does not cover every HPV type and screening still matters.** ● Keep using condoms. This vaccine protects against HPV, not against other sexually transmitted infections or pregnancy. ● A sore arm, headache, mild fever or tiredness for a day or two is common and settles by itself. ● Some people feel faint around injections. That is why we ask you to

	stay seated with us for 15 minutes. ● **If you are eligible for this free on the NHS, we will tell you, and you are free to go that route instead.**
Follow-up	Where a further dose is due, book it at this appointment and give the date in writing. Seek medical advice for any severe or persistent reaction, for any rash or swelling that spreads, or for any breathing difficulty. For routine queries contact the pharmacy.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions.
- Immunisation against infectious disease (the Green Book), chapter 18a, Human papillomavirus (HPV), UK Health Security Agency, dated 19 June 2023 (GOV.UK last updated 20 June 2023). Summarised 9 September 2026.
- The current Summary of Product Characteristics for each product named in this PGD, medicines.org.uk.
- British National Formulary, current edition.

Authorisation of this version

Authorised by the Medical Director and the Head Pharmacist named below. This version is not valid without both signatures.

Version	v008
Supersedes	Version 007, 14 September 2026
Valid from date	30 September 2026
Expiry date	31 July 2027

Authorised on behalf of Get Real Health by the Medical Director and the Head Pharmacist. Their signatures for this version are on the signed authorisation page at the end of this document.	

PGD adoption and authorisation by the employer or clinical lead

The employer has ensured that the person using this PGD has the knowledge and skills to safely provide the service, and that appropriate governance is in place. This section must be signed by the superintendent or clinical lead responsible for this service.

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....
Professional group signatory	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....

Agreement to practise by the registered healthcare professional

By signing below you confirm that you agree to the contents of this PGD and that you will work within it. A PGD does not remove the inherent professional obligations or accountability of the individual practitioner. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of the pharmacy or clinic premises to which this PGD relates

Premises name	.....
Address	.... Postcode: .....
ODS code, if applicable	.....

Registered healthcare professionals authorised to work under this PGD at those premises

The employing organisation must keep this page, or an equivalent local register, and must be able to produce it on request. A practitioner who has not signed against the current version is not authorised to work under it.

Name of healthcare professional	Job title | Registration number | Signature | Date
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Valid from date	30 September 2026
Expiry date	31 July 2027

GET REAL HEALTH

Change history to version 003

Later versions are recorded in the Version and change record at the end of this document.

Version	v003
Date	9 September 2026
Changes	● The practitioner Agreement to practise page, the premises block and the practitioner signature table are restored, together with the standard governance requirements: SPC and BNF familiarity, MHRA safety alerts, CPD and appraisal, indemnity, and capacity and consent. Every version 001 document carried them; no document produced by the current generator did, through one omission in one function that affected 15 live documents. Raised as blocking by an adopting pharmacy. This version changes no clinical content. ● The document and the consultation tool are corrected to cover all sexes. Version 001 described the national cohorts correctly, including GBMSM up to 45 years of age, but the consultation tool required a "female confirmed" tick before it would proceed and would not produce a dose recommendation for a male patient at all. A PGD that names the GBMSM cohort while its tool refuses to vaccinate men is not a service; the tool has been rebuilt alongside this version. ● Yeast allergy removed as a contraindication. Green Book chapter 18a states that yeast allergy does not contraindicate HPV vaccine because the finished product contains no yeast as an ingredient. Version 001 listed yeast protein among its contraindications and the tool applied a hard stop to it, so patients were being refused a vaccine they could safely have. ● The schedule is now selected from age and immune status, and is stated once instead of twice. Version 001 gave the JCVI schedules under "dose and frequency" and the SPC schedules under "maximum or minimum treatment period", so the document set out two incompatible courses and never said which to follow. The tool, meanwhile, recommended three doses at 0, 2 and 6 months to every patient regardless of age, which meant an immunocompetent person under 25 was being asked to return, and to pay, for two doses that national policy says they do not need. ● The one dose and two dose schedules are identified as off-label, with a stated consent script and a requirement to record consent to off-label use naming the schedule. The Gardasil 9 SPC contains no one dose schedule and requires three doses from 15 years of age; the UK schedules come from JCVI. Green Book chapter 18a recommends them without flagging their licensing status, so a pharmacist relying on the chapter alone would not know. ● Immune status must now be established before the schedule is chosen, and recorded. Version 001 gave a three dose schedule for immunosuppressed patients but never required anyone to ask. ● Pregnancy handling corrected. Green Book chapter 18a states that routine questioning about last menstrual period and pregnancy testing is not required, and that termination must not be recommended after inadvertent immunisation. Version 001 excluded pregnancy without either statement, and the tool told the pharmacist to "counsel on avoiding conception during the course", which has no basis in the guidance and would alarm a patient for no reason. ● A requirement for adrenaline 1 in 1,000 to be immediately available in the room, with a written anaphylaxis protocol and a telephone, and for staff to be trained in anaphylaxis management and hold current basic life support. Version 001 carried the SPC boilerplate that treatment should be "readily available" and no requirement of any kind. ● A 15 minute observation period after every vaccination, seated, stated

	as a requirement rather than mentioned among the cautions. ● A cold chain excursion procedure added, using the concrete SPC stability figures of 96 hours at 8C to 40C or 72 hours at 0C to 2C, together with quarantine and risk assessment under UKHSA Vaccine Incident Guidance. Version 001 had none. ● A sharps disposal provision added, referencing HTM 07-01. Version 001 had none. ● Consent in children and young people added: parental responsibility, or Gillick competence with the basis recorded. This PGD starts at 9 years of age and its largest cohort is 12 to 13 year olds, yet version 001 said only "informed consent obtained or parental/guardian consent as appropriate". Found by an estate-wide sweep of every vaccination PGD on 8 September 2026, which flagged the same gap in nine documents. ● Bleeding disorder guidance expanded from a bare caution to the Green Book chapter 18a practical instruction: 23 gauge or finer needle, firm pressure without rubbing for at least two minutes, and the position on haemophilia treatment and stable warfarin. ● Records must now carry immune status, the schedule selected and why, the batch number, the dose number and next dose date or a statement that the course is complete, the site of each vaccine where more than one was given, that the observation period was completed, and that the patient was told whether they are NHS-eligible. ● The requirement to tell every patient whether they could have the vaccine free on the NHS is moved from a passing clause in the inclusion criteria into an explicit recorded step, with the eligibility cohorts set out in an appendix.

Previous versions

002, 8 September 2026	All sexes covered and the tool rebuilt; yeast allergy removed as a contraindication; schedule selected from age and immune status; the one dose and two dose schedules identified as off-label.
001, 16 February 2026	Development and issue of new PGD. Gardasil 9 from 9 years of age.

Appendix 1: NHS eligibility, for the conversation you must have

Every patient must be told whether they could get this vaccine free on the NHS. This appendix is the reference for that conversation. It is not a list of who may be vaccinated under this PGD: this PGD covers anyone aged 9 and over who meets the inclusion criteria, NHS-eligible or not.

- All adolescents, of any sex, from 11 years of age, offered in school year 8 in England and Wales, S1 in Scotland and year 9 in Northern Ireland.
- Individuals in those eligible cohorts remain eligible until their 25th birthday. For England the cohorts are females born after 1 September 1991 and males born after 1 September 2006. Cut-offs differ in Scotland, Wales and Northern Ireland.
- Those cohorts include people who have moved to the UK from overseas, are registered with a GP practice, are under 25, and were not offered HPV vaccination in their country of origin.
- All GBMSM up to and including 45 years of age attending specialist sexual health services or HIV services, regardless of risk, behaviour or disease status.
- Anyone who started but did not complete the GBMSM course before reaching 46 should be allowed to complete it.
- Some transgender individuals, sex workers and people living with HIV attending those services, where their risk is judged equivalent to that of eligible GBMSM.
- **Children aged 9 to 11 are licensed for the vaccine but are NOT covered by the national programme. A private supply under this PGD is their only route.**

Appendix 2: Key references

- Immunisation Against Infectious Disease (the Green Book), chapter 18a, Human papillomavirus, June 2023, incorporating the move to a one dose schedule from 1 September 2023.
- Immunisation Against Infectious Disease (the Green Book), chapter 8, Vaccine safety and the management of adverse events, for the management of anaphylaxis and the distinction between a faint, a panic attack and anaphylaxis.
- Summary of Product Characteristics for Gardasil 9, Merck Sharp and Dohme (UK) Limited, PLGB 53095/0023, text last revised 13 September 2024.
- JCVI advice on the HPV immunisation programme, and the bipartite letter of 20 June 2023 on the change to a one dose schedule from September 2023.
- UKHSA Vaccine Incident Guidance, for cold chain excursions.
- Health Technical Memorandum 07-01, Safe management of healthcare waste.
- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions, https://www.nice.org.uk/guidance/mpg2

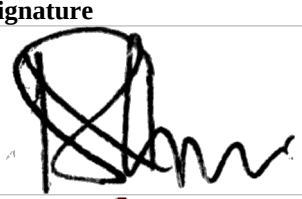
Version and change record
Version: v008
Issued: 30 September 2026
Supersedes: Version 007, 14 September 2026
Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records
Version: v007
Issued: 14 September 2026
Supersedes: Version 006, 11 September 2026
Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.
Previous version records
Version: v006
Issued: 11 September 2026
Supersedes: Version 005, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Decision 17: a single dose received before the 25th birthday completes the course for an immunocompetent patient only. An immunosuppressed or HIV positive patient completes the three dose course whatever their age at the first dose, with the earlier dose counted and not repeated. The schedule bullets, exclusion criteria, cautions, dose and frequency row and the guidance summary now say the same thing (Green Book chapter 18a).
Decisions of the authorising signatories, 11 September 2026: the questions raised by the reviews of 10 and 11 September were put to the Medical Director and answered; each answer is written into this document above and, where the answer set a rule, into the consultation tool. Text drafted from a source for the Head Pharmacist to confirm is marked as such in the change lines.
Previous version records
Version: v005
Issued: 11 September 2026
Supersedes: Version 004, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Cover and arm titles corrected from 'for the treatment of Prevention of' to 'for the prevention of' HPV-related cancers and genital warts.
Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.
Previous version records
Version: v004
Issued: 11 September 2026
Supersedes: Version 003, 8 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Signed on behalf of Get Real Health

Version v008, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.